



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سكينه عوض الحفني حسن
المحافظة : الشرقيه
الحى / المدينة : العاشر من
رمضان
رقم البطاقة : 28512021303689 رقم التليفون : 01000828370
العمر (تاريخ الميلاد) : 02-12-1985 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 04-10-2026
العنوان : العاشر

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سكينه عوض الحفني حسن National ID: 28512021303689	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
فضولت ونهال





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: سكرية مرض الجليم حسن

Sex:

MDT form

3- Medical number:
Female

83167629

Old Med:

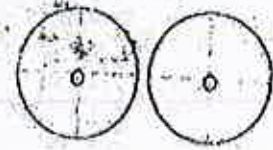
REVIEW:

3971/2023

2-ID number: 28512021303689

Patient Age: 39 Y

Post



Family history

Family history

+VE F.H aunt

Phone

Phone

0100002837

DNI

64

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

MARRIED

NRS Score

NRS Score

GROUP

GROUP

A

Date of MDT

Date of MDT

05/06/2024

Co morbidity

Co Morbidity

free

Menopausal
Status

Menopausal Status
PREMENOPAUSE

Initial STAGE

PRE-STAGE

Clinical data

Breast Imaging :

LT pathological proved breast + path L.N.

Mets Workup :

CT : FREE

BX

IDC

EX LUHENT

ER

8/8

EX COMMENT

PR

8/8

EX COMMENT

Her 2 Neu

0

Her 2 Neu COMMENT

SYSH

SISTEMIC

Ki67

40

T

2

N

2

M

0

Sonomammography

PATIENT HISTORY

LT WLE + ALND:IDV GII

free skin

free margin

LN 3/14

MDT OUTCOME & PLAN

in view of having YpT2N1 for :

1- PORT

2-aromatase inhibitors/zoladex

3-trial of abemaciclib

MEDICAL ONCOLOGY

SURGERY

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

Signature

Signature

Signature

Signature

Colleg
Hiba

Patient Name : الأستاذة / سكرينة مريض الحثاني حسن
Patient No. : 28512021303689
Path No. : 3316/23
Sex : Female
Age : 37 Y
Referring Doc :

Request No. : 13000541
Request Date : 27/09/2023
Verification Date : 09/10/2023
Financial Category : مبالغه - مبالغه
Department : Out Patient
Registered by : Mrs. Aya Mohamed

Pathology Report

Clinical data
Left breast mass

Nature of specimen

Tru cut biopsy, collected at 27/9/2023, 10:30 am

Gross examination

1) Left lateral swelling

Multiple (3) grayish white and fatty tissue cores up to 1.5 cm, totally submitted.

2) Left axillary tail lesion:

Multiple (3) grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic examination

* Histological type: Invasive duct carcinoma with cribriform differentiation.

* Grade (differentiation + nuclear pleomorphism + mitosis): (2+2+3) Grade II

* DCIS: No

* Angiolysis: Couldn't be detected in the received biopsy

* Perineural invasion: No

2) Left axillary tail swelling:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation + nuclear pleomorphism + mitosis): (2+2+3) Grade II

* DCIS: No

* Angiolysis: Couldn't be detected in the received biopsy

* Perineural invasion: Positive

Diagnosis

Left breast masses, Tru cut biopsy:

1) Left lateral swelling:

Verified by :

Dr. Hiba Hasan

Printing Date : 11/10/2023 10:26:24

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ملف المريض : 28512021303689 - رقم الملف : 3316/23 - تاريخ التسجيل : 27/09/2023 - رقم الملف : 3316/23 - رقم الملف : 3316/23
ملف المريض : 28512021303689 - رقم الملف : 3316/23 - تاريخ التسجيل : 27/09/2023 - رقم الملف : 3316/23 - رقم الملف : 3316/23

Patient Name : الأستاذة / مكيولة خورشيد العالي حنين
Patient No. : 28512021303689
Path No. : 3316/23
Sex : Female
Age : 37 Y
Referring Doc :
Request No. : 13000541
Request Date : 27/09/2023
Verification Date : 09/10/2023
Financial Category : مجاني - مجاني
Department : Out Patient
Registered by : Mrs. Aya Mohamed

Pathology Report

Invasive duct carcinoma, Grade II with cribriform differentiation
For marker study (ER, PR, HER2, Ki67)
2) Left axillary tail swelling
Invasive duct carcinoma, Grade II.
For marker study (ER, PR, HER2, Ki67)

Signature
Prof. Dr. Iman Gouda
Dr. Heba Hassan

Verified by :
Dr. Heba Hassan

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ملحق : 23-10-2023
المرجع : 28512021303689
المرجع : 3316/23
المرجع : 13000541
المرجع : 27/09/2023
المرجع : 09/10/2023
المرجع : مجاني - مجاني
المرجع : Out Patient
المرجع : Mrs. Aya Mohamed



Patient Name: Sekina Awad Al Hefny Hassan	Date: 14/10/2023
Exam: MAMMOGRAPHY BOTH	Age: 37 Year(s)
Medical ID: 38	Pat. ID: 42979

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Left breast pathologically proved malignant mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Left axillary tail focal asymmetry. US showed two spiculated hypoechoic solid lesions measuring 2.6x2.1 cm, 0.25 cm from the overlying skin and 1.4x1.2 cm, 0.5 cm from the overlying skin, 1.4 cm apart.
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Right Nonspecific axillary LNs.
- Left two pathologically enlarged axillary lymph nodes showing diffuse cortical thickening and lost hilum seen in level I

Opinion:

- Normal right breast study (BIRADS 1)
- Left UOQ two pathologically proved malignant masses with ipsilateral two pathological axillary lymph nodes level I (BIRADS 6)

Yours truly

Signature Dr. Yasmin Nada

14/10/2023 2:19 PM



Patient Name: Sekina Awad Al Hefny Hassan	Date : 18/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 39 Year(s)
Medical ID: 3871/2023	Pat. ID: 42979

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN,
AND PELVIS: -**

Findings:

I Chest:-

Compared to previous non contrast study dated 16/12/2024

- Status post left BCS showing operative bed distortion and skin thickening.
- stable few bilateral scattered tiny pulmonary nodules.
- Regressive course of the prominent superior mediastinal soft tissue mostly thymic.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- Cholecystectomy clips.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

OPINION:

- Status post left BCS showing the above described changes.

Dr. Mohamed Mohsen, M.D.

Patient Name	: الأستاذة / مكيته عروضة الحلفي حسن	Request No.	: 13000541
Patient No.	: 28512021303689	Request Date	: 27/09/2023
Path No.	: 3316/23	Verification Date	: 25/10/2023
Sex	: Female	Financial Category	: برنامج دار السلام - مبدئية
Age	: 37 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs. Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass
Nature of Specimen: Tru-cut biopsy.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT of both masses (1&2) (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
PR: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8).
Her2: No membrane reaction in tumor cells (score 0).
Ki67 of left lateral swelling #1: 40% nuclear reaction of tumor cells.
Ki67 of left axillary tail #2: 20% nuclear reaction of tumor cells.

Conclusion:

Left breast masses, Tru-cut biopsy:

1- Left lateral swelling:

Invasive duct carcinoma, Grade II with cribriform differentiation.

PT:

ER: Positive (score 8/8).

PR: Positive (score 8/8).

Her2: Negative (score 0).

Ki67: Positive in 40% of tumor cells.

2- Left axillary tail swelling:

Verified by:

Dr. Esraa Magdy

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CAP

عنوان رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Min El Qalena - Cairo, Egypt

Patient Name: الأستاذ / سكينه عوض الحفنى حسن
Patient No.: 28512021303689
Path No.: 3316/23
Sex: Female
Age: 37 Y
Referring Doc.:

Request No.: 13000541
Request Date: 27/09/2023
Verification Date: 25/10/2023
Financial Category: اورام دار السلام - مبادره
Department: Out Patient

Registered by: Mrs. Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

Invasive duct carcinoma, Grade II.

PT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is intense and within < 10% of the invasive tumor cells.

Signature
Prof. Dr. Iman Gouda
Dr. Menina Nabil

Verified by:
dr.esraa magdy

Printing Date : 25/10/2023 12:04:25

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شرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

???, ????? Patient ID: 10110225002392 DOB: 21-Dec-1985 Date: 06-Feb-2025

Patient Lastname: عوض

Patient Firstname: سكينة

HIS Patient ID: 10110225002392

RIS Patient ID: 366047

Date of Birth: 21/12/1985

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (6/2/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

- " No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====